

HIGHLIGHTS OF PRESCRIBING INFORMATION: LISINOPRIL TABLETS

WARNING: FETAL TOXICITY

- When pregnancy is detected, discontinue lisinopril as soon as possible.
- Drugs that act directly on the renin-angiotensin system can cause injury and death to the developing fetus. See Warnings and Precautions.

1 INDICATIONS AND USAGE

Lisinopril is an ACE inhibitor indicated for:

- Treatment of hypertension in adult patients and pediatric patients 6 years of age and older to lower blood pressure.
- Adjunctive therapy in the management of heart failure in patients who are not responding adequately to diuretics and digitalis.
- Treatment of hemodynamically stable patients within 24 hours of acute myocardial infarction to improve survival.

2 DOSAGE AND ADMINISTRATION

- Hypertension: Initial adult monotherapy dose is 10 mg once daily. Adjust dosage according to blood pressure response up to maximum 40 mg once daily.
- Heart Failure: Initial dose 2.5 mg to 5 mg once daily with diuretics and digitalis. Target dose is 20 mg to 40 mg once daily.
- Acute Myocardial Infarction: 5 mg within 24 hours, followed by 5 mg after 24 hours, 10 mg after 48 hours, then 10 mg daily for 6 weeks.
- Renal Impairment: For CrCl 10 to 30 mL/min, initial dose is 5 mg daily. For CrCl < 10 mL/min, initial dose is 2.5 mg daily.

4 CONTRAINDICATIONS

- Lisinopril is contraindicated in patients with a history of angioedema related to previous ACE inhibitor treatment, or hereditary or idiopathic angioedema.
- Do not coadminister aliskiren with lisinopril in patients with diabetes.
- Do not coadminister with a neprilysin inhibitor (e.g., sacubitril). Allow 36 hours between administration of lisinopril and sacubitril/valsartan.

5 WARNINGS AND PRECAUTIONS

- Fetal Toxicity: See Boxed Warning.
- Angioedema: Head and neck angioedema, including laryngeal edema, may occur and be fatal. Intestinal angioedema presenting with abdominal pain has also been reported. Discontinue lisinopril immediately and administer emergency therapy.
- Hypotension: Symptomatic hypotension may occur, particularly in volume-depleted or salt-depleted patients.
- Renal Impairment and Hyperkalemia: Monitor renal function and serum potassium periodically. Hyperkalemia risk is increased in patients with renal impairment, diabetes, and those using potassium supplements or potassium-sparing diuretics.
- Persistent Dry Cough: Characteristically nonproductive, persistent, and resolves after cessation of therapy.

6 ADVERSE REACTIONS

Most common adverse reactions (incidence $\geq 1\%$): Headache, dizziness, persistent dry cough, fatigue, nausea, diarrhea, rash, orthostatic effects, and hyperkalemia.

7 DRUG INTERACTIONS